AS 85000:2024

Australian STANDARD®

Australian Community Pharmacy Standard



The Pharmacy Guild of Australia

Originated as AS 85000:2011

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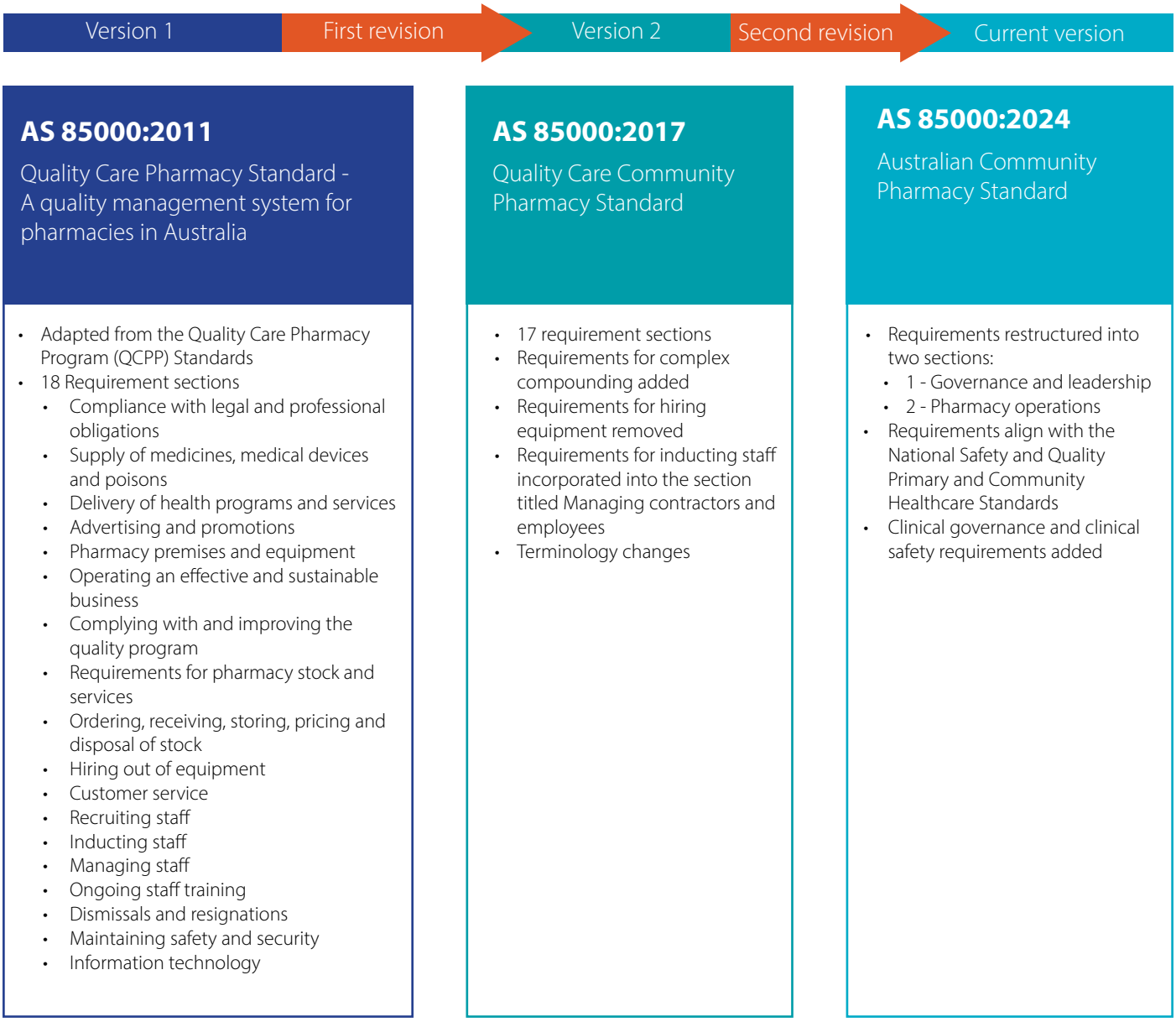
AS 85000: 2024 Australian Community Pharmacy Standard

Preface

The Australian Standard® (AS) 85000:2024 outlines the requirements for a quality management system for Australian community pharmacies. The requirements set out in this standard support the provision of safe and consistent pharmacy services that contribute to improved health outcomes for Australian patients, consumers and communities.

AS 85000:2024 supersedes the previous version of this standard, AS 85000:2017 *Quality Care Community Pharmacy Standard*. The first version of this standard was AS 85000:2011 *Quality Care Pharmacy Standard: A quality management system for pharmacies in Australia*.

The following diagram shows the main differences between versions of AS 85000.



The Pharmacy Guild of Australia (PGA) is an accredited Standards Development Organisation under Standards Australia and is the custodian of this standard. PGA led the development of this standard using an open process of consultation and consensus in which all key stakeholders were invited to participate.

A Standards Development Committee was established to oversee the review of AS 85000:2017 and development of AS 85000:2024. The following organisations were represented on the Standards Development Committee:

- Australian Association of Consultant Pharmacy
- Australian Commission on Safety and Quality in Health Care
- Australian Patients Association
- Australian Pharmacy Council
- GuildLink
- MedAdvisor
- National Aboriginal Community Controlled Health Organisation
- National Australian Pharmacy Students' Association
- Pharmaceutical Defence Limited
- Pharmaceutical Society of Australia
- Pharmacy Board of Australia under the Australian Health Practitioner Regulation Authority
- Pharmacy Guild of Australia
- Professional Compounding Chemists of Australia
- Professional Pharmacists Australia
- RSM Australia Pty Ltd

A Standards Reference Group of subject matter experts was also convened to provide detailed advice and guidance on the content for the revised standard. The following organisations were represented on the Standards Reference Group:

- Consumer Health Products Australia
- GuildLink
- MedAdvisor
- Medical Software Industry Association
- National Aboriginal Community Controlled Health Organisation
- Pharmaceutical Defence Limited
- Pharmaceutical Society of Australia
- Pharmacy Banner Groups
- Pharmacy Guild of Australia
- Pharmacy Premises Registration Authorities of Australia
- Professional Compounding Chemists of Australia
- Professional Pharmacists Australia
- RSM Australia Pty Ltd
- Rural Pharmacists Australia
- Therapeutic Goods Administration

The PGA wishes to acknowledge the participation of the expert individuals that contributed to the development of this standard through their representation on the committees above and through the public comment period.

This standard was approved by the PGA Clinical Governance Committee on 9 May 2024.

This standard was issued in draft form as AS 85000 Australian Community Pharmacy Standard (Draft) on 27 September 2023.

This standard was published in June 2024.

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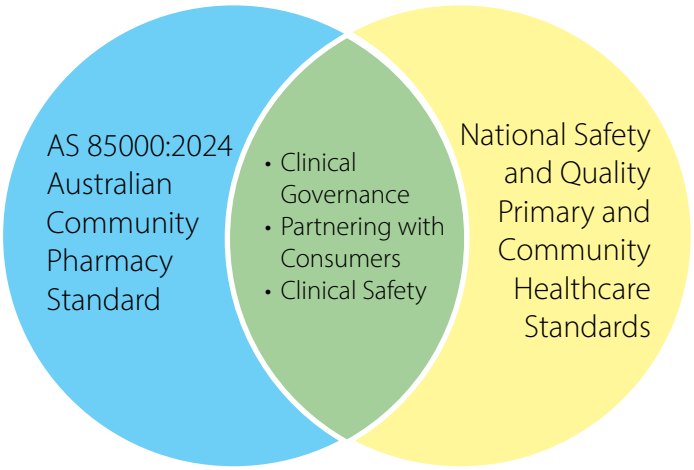
Introduction

The 2024 version of this Australian Standard® (AS 85000:2024) has been developed through an open, consultative review process. The breadth of community pharmacy practice in Australia has seen significant growth and technological advances in the years since the last version of this standard was published in 2017. Examples of the expansion of practice include the administration of vaccines by pharmacists, the use of electronic prescriptions and prescribing by pharmacists. The review of the standard provided an opportunity to ensure it reflects current practice and can continue to support further advances in practice into the future.

In addition, the Australian Commission on Safety and Quality in Health Care (the Commission) published the *National Safety and Quality Primary and Community Healthcare Standards* (Primary and Community Healthcare Standards) in 2021. These standards describe the processes and structures needed to deliver safe and quality health care and provide a nationally consistent framework applicable to all primary and community healthcare services.

Alignment with the Primary and Community Healthcare Standards was a high priority during the revision process to ensure community pharmacies are contributing to the overall healthcare system in a consistent, safe, and quality manner. PGA has worked closely with the Commission to ensure AS 85000:2024 is aligned with the Primary and Community Healthcare Standards.

Diagram: Alignment between AS 85000:2024 and the National Safety and Quality Primary and Community Healthcare Standards



AS 85000:2024 *Australian Community Pharmacy Standard* sets the standard for the accreditation program for community pharmacies in Australia. Accredited pharmacies are recognised as demonstrating:

- a commitment to quality care and continuous quality improvement,
- delivery of consistent, safe and quality pharmacy services, and
- robust clinical and business governance systems.

Accredited pharmacies will have demonstrated they meet the requirements of this standard and, as such, have a quality management system and clinical governance framework in place. In addition, their systems and processes will be aligned with the Primary and Community Healthcare Standards.

Australian Standards are living documents that reflect progress in science, technology and systems. To maintain their currency, all standards are periodically reviewed and new editions are published. Between editions, amendments may be issued and standards may also be withdrawn. It is important that readers assure themselves that they are using a current standard, which should include any amendments that may have been issued since the standard was published.

The PGA welcomes suggestions for improvements and encourages readers to notify us immediately of any apparent inaccuracies or ambiguities. Contact us via email at guild.nat@guild.org.au

1. Scope

AS 85000:2024 *Australian Community Pharmacy Standard* provides guidance for the design, management and continuous improvement of a quality management system for community pharmacies in Australia. The standard contains requirements for the following areas:

1. Pharmacy governance and leadership, including clinical governance and business governance principles.
2. Partnering with patients and consumers in their health care.
3. Principles for the design, delivery and continuous improvement of pharmacy services.
4. The safe and appropriate provision of medicines from community pharmacies.
5. Clinical safety principles in pharmacy.
6. The successful operation of pharmacy businesses.

This standard does not apply to the hospital pharmacy setting.

2. Application

AS 85000:2024 *Australian Community Pharmacy Standard* is the standard against which community pharmacies are assessed to gain quality accreditation in Australia. When preparing for accreditation, pharmacy owners and staff should read this standard in conjunction with the pharmacy accreditation requirements and terms and conditions to gain a full understanding of the actions they need to complete, and evidence they need provide, to gain accreditation.

The clauses of this standard are grouped into two main sections:

SECTION 1 – Governance and Leadership
SECTION 2 – Pharmacy Operations.

Each clause is followed by subclauses that divide the information into smaller topics and contain the actions required to meet the standard.

<table>
<tr><th colspan="4">SECTION 1 – Governance and Leadership</th></tr>
<tr><td colspan="2">Clause 1 – Clinical Governance</td><td colspan="2">Clause 2 – Business Governance</td></tr>
<tr><td colspan="2">• Governance, leadership and culture
• Patient safety and quality systems
• Clinical performance and effectiveness
• Safe environment for the delivery of care
• Partnering with patients in their own care
• Health literacy
• Partnering with consumers in pharmacy design and governance</td><td colspan="2">• Legal and professional obligations
• Business viability
• Business continuity plan
• Pharmacy premises
• Safety and security
• Information technology</td></tr>
<tr><th colspan="4">SECTION 2 – Pharmacy Operations</th></tr>
<tr><td>Clause 1 – Pharmacy services</td><td>Clause 2 – Safe and appropriate provision of medicines</td><td>Clause 3 – Clinical safety</td><td>Clause 4 – Business operations</td></tr>
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• Complex compounding
• Indirect provision of medicines
• Advertising of therapeutic goods</td><td>• Preventing and controlling infections
• Comprehensive care
• Communicating for safety
• Responding to serious deterioration and minimising harm</td><td>• Human resources
• Safety and security
• Information technology</td></tr>
</table>

SECTION 1 – Governance and Leadership

Section 1 of the standard contains clauses, subclauses and actions covering governance and leadership principles, including clinical governance and business governance.

Clinical governance describes the accountability of pharmacy owners in ensuring that there is a strong culture and appropriate systems and processes in place to support the safety, quality and continuous improvement of all activities and services provided in, or from, a community pharmacy. It involves relevant and appropriate relationships and responsibilities to be established between pharmacy owners, their staff, patients, consumers and other stakeholders to support improved clinical outcomes for patients. It ensures that community pharmacies and their patients can be confident that systems are in place to deliver safe, quality and patient-centred health care.

The clinical governance actions set out in this section have been adapted from the Primary and Community Healthcare Standards to ensure there is alignment, consistency and integration between community pharmacy services and other primary health services being delivered across Australia.

To create an environment in which quality health services can be delivered, a pharmacy needs to operate as a successful business. To achieve this, business planning and strategic thinking are as important as financial budgeting and reporting. A documented business plan helps with measuring progress and achieving goals and objectives. Business risks need to be managed, just as clinical risks do, to ensure the pharmacy is sustainable and can continue to provide medicines and services to its community.

It is the responsibility of pharmacy owners to define the business governance structure of the pharmacy, relative to its own unique requirements. Governance arrangements will vary depending on the size and complexity of each pharmacy. Smaller pharmacies may have a merging of governance and management responsibilities, whereas other pharmacies may be part of a wider corporate group.

Pharmacy owners have a responsibility to define clinical and business governance structures applicable to their pharmacies, to implement systems and processes to support improved clinical outcomes and business sustainability, and to oversee and drive these outcomes in their pharmacies. These governance principles underpin all other activities conducted in the pharmacy and it is imperative these foundations are in place.

SECTION 2 – Pharmacy Operations

Section 2 of this standard outlines core pharmacy functions and their operational requirements. There are 4 clauses in this section, which cover the delivery of pharmacy services, the safe and appropriate provision of medicines, clinical safety, and business operations. Pharmacies are required to embed the foundational systems and processes from Section 1 Governance and Leadership to support the implementation of the actions outlined in Section 2.

Pharmacies across Australia provide a large number of services to their communities, ranging from staged supply of medicines to dose administration aids and vaccination services. The clauses and actions relating to pharmacy services outline the principles that support the delivery of safe, high-quality pharmacy services. These principles apply to all types of services, from the simple to the more comprehensive and those that may emerge in the future. Detailed, service-specific requirements that complement the standard can be found in professional practice guidelines or program rules.

In addition to the services they provide, pharmacies offer a large range of medicines and other health-related products and must ensure these items are stored, displayed and provided in a manner that maintains the integrity of the items and supports the safe and quality use of medicines. In addition, legislative requirements and professional practice standards and guidelines relating to the storage, handling and provision of medicines and therapeutic products must be met.

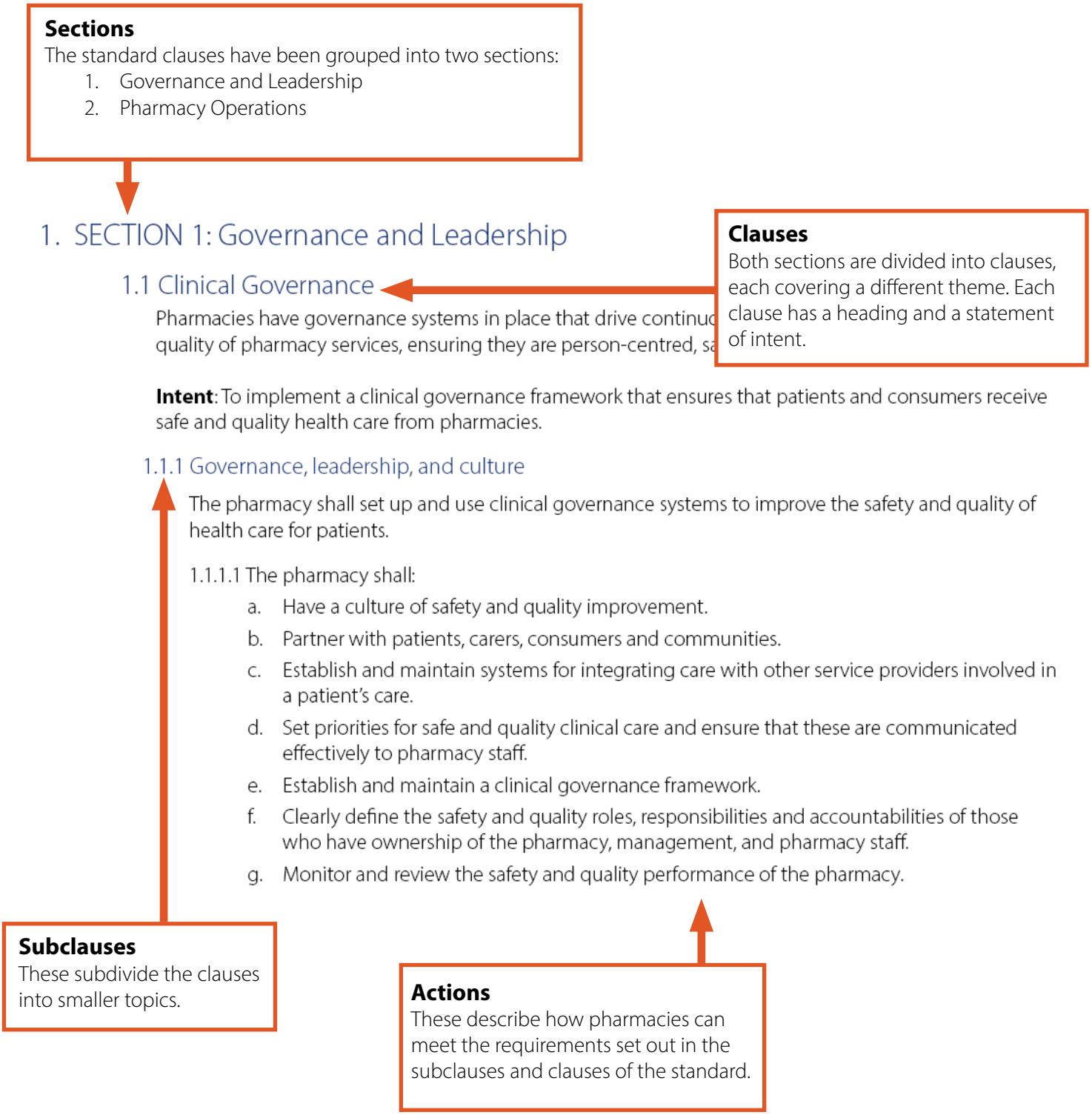
The clinical safety clauses and actions address risks to safety and quality that are commonly encountered in pharmacy and other primary health care settings, such as the spread of infection and a lack of coordination in the delivery of health care. The actions support a collaborative and integrated approach to patient care, and the implementation of

systems to facilitate effective communication between healthcare professionals, as well as recognising and responding to potentially harmful situations.

To support the provision of safe and quality pharmacy services and therapeutic goods, pharmacies must have knowledgeable pharmacy staff and secure information systems. The business operations clauses and actions outline the human resource and information technology requirements to ensure the successful day-to-day operation of a pharmacy. They also outline the importance of identifying and preventing risks to the pharmacy, including dangerous situations and emergencies.

How to read the standard

The following diagram shows the different levels of information presented in this standard.



While reading this standard, you will notice the use of the following verbal forms indicating whether a clause is a mandatory requirement or a recommendation. Notes are also used throughout the standard to provide additional information about the intent and context of different clauses.

	REQUIREMENT	RECOMMENDATION	NOTES
Verb form	Shall / Shall not	Should / Should not	NOTE:
Meaning	Indicates the statement is mandatory	Indicates a recommendation	Indicates supplementary information
Example	*The pharmacy shall implement, maintain and follow a process to recover and restore IT systems and data in the event of infrastructure loss, or loss of system access.*	*The pharmacy should have access to a defibrillator and ensure pharmacy staff are able to use the defibrillator in the case of an emergency.*	*NOTE: Information gathered from patient populations should be gained with appropriate consent.*

3. Normative references

The following documents are referred to in the text in such a way that some, or all, or part of their content constitutes requirements of this document.

- Australian Commission on Safety and Quality in Health Care's National Safety and Quality Primary and Community Healthcare Standards [i]
- Australian Open Disclosure Framework [ii]
- Australian Charter of Healthcare Rights [iii]
- State and Territory SafeWork's Compliance Code: Workplace Amenities and Work Environment
- Therapeutic Goods Administration's Therapeutic Goods Advertising Code [iv]
- National Vaccine Storage Guidelines 'Strive for 5' [v]
- Australian Register of Therapeutic Goods
- National Hand Hygiene Initiative
- Australian Immunisation Handbook [vi]
- Antimicrobial Stewardship Clinical Care Standard [vii]

4. Terms and definitions

4.1 Adverse Drug Reaction – a response to a medicine that is noxious and unintended and occurs at doses normally used or tested in humans for the prophylaxis, diagnosis or therapy of disease, or for the modification of physiological function.[i]

4.2 Advertising – in relation to therapeutic goods, includes making any statement, pictorial representation or design that is intended, whether directly or indirectly, to promote the use or supply of the goods. Advertising can occur through many different mediums, including (but not limited to) websites, social media, print media, radio or television. [viii]

4.3 Antimicrobial stewardship – an ongoing effort by healthcare services to reduce the risks associated with increasing antimicrobial resistance and to extend the effectiveness of antimicrobial treatments. It may incorporate several strategies, including monitoring and review of antimicrobial use.[i]

4.4 Aseptic technique – a set of practices aimed at minimising contamination, used particularly to protect patients from infection during procedures.[i]

4.5 Australian Charter of Healthcare Rights – a document that specifies the key rights of patients and consumers when seeking or receiving healthcare services[i]

4.6 Business Continuity Plan – a plan to help a pharmacy respond to a crisis event, minimise interruption, recover and resume normal operations as quickly as possible. A crisis event may be a fire, flood, loss of key staff, pandemic etc.

4.7 Business governance – the set of relationships and responsibilities established by the pharmacy between its owners, management, staff and stakeholders (including patients and consumers) to ensure the pharmacy business is viable and sustainable and operates in compliance with relevant laws and regulations.

4.8 Clinical governance – the accountability of pharmacy owners in ensuring that there is a strong culture and appropriate systems and processes in place to support the safety, quality and continuous improvement of all activities and services provided in or from a community pharmacy. It involves relevant and appropriate relationships and responsibilities to be established between pharmacy owners, their staff, patients, consumers and other stakeholders to support good clinical outcomes for patients. It ensures that community pharmacies and their patients can be confident that systems are in place to deliver safe, quality and patient-centred health care.

4.9 Clinical performance – the ability of pharmacy staff to deliver safe, quality healthcare and achieve appropriate clinical outcomes.

4.10 Cold chain management – the system of transporting and storing temperature-sensitive medicines and vaccines within their defined temperature range at all times, from point of origin (manufacture) to point of administration, to ensure that the integrity of the product is maintained.[i]

4.11 Communities – groups of people that may or may not be spatially connected but who share common interests, concerns or identities. Communities can be local, national or international and exist at multiple levels; there can be communities within communities.[ix]

4.12 Community pharmacy – also known as a retail pharmacy, a service that provides pharmaceutical products, services, and health advice in the community setting.

4.13 Compounding – the extemporaneous preparation and supply of a single 'unit of issue' of a therapeutic product intended for supply for a specific patient in response to an identified need. The practice of compounding is classified as either simple or complex compounding. Compounding/manufacturing may also be defined in State and Territory legislation.[x]

4.14 Comprehensive care – health care that is based on identified goals for the episode of care. These goals are aligned with the patient's expressed preferences and healthcare needs, consider the impact of the patient's health issues on their life and wellbeing, and are clinically appropriate.[i]

4.15 Consumer – a person who has used, or may potentially use, pharmacy services, or is a carer or family member of a patient using pharmacy services.[i]

4.16 Critical information– information that has a considerable impact on a patient's health, wellbeing or ongoing care (physical or psychological). The availability of critical information may require a healthcare provider to reassess or change a patient's comprehensive care plan.[i]

4.17 Critical medical equipment – instruments, equipment and devices that confer a high risk for infection if they are contaminated with microorganisms and that must be sterile at the time of use. They include any objects that enter sterile tissue or the vascular system, as any microbial contamination could transmit disease.[i]

4.18 Cultural safety – culturally safe practice is the ongoing critical reflection of health practitioner knowledge, skills, attitudes, practising behaviours and power differentials in delivering safe, accessible and responsive healthcare free of racism. Culturally safe practice should apply to Aboriginal people, Torres Strait Islanders and people from diverse backgrounds (see 4.20). Cultural safety is determined by Aboriginal and Torres Strait Islander individuals, families and communities.[xi]

4.19 Diagnosis – the identification, by a health professional within their scope of practice, of a condition, disease or injury, made by evaluating the symptoms and signs presented by a patient.[xii]

4.20 Dispensing – the review of a prescription and the preparation, packaging, labelling, record keeping and transfer of the prescribed medicine, including counselling, to a patient, their agent or another person who is responsible for the administration of the medicine to that patient.[xiii]

4.21 Diverse backgrounds – the varying social, economic and geographic circumstances of consumers who use, or may use, the services of a healthcare service, as well as their cultural backgrounds, disability status, religions, beliefs and practices, languages spoken, sexual orientation, gender identity and gender expression, and sex characteristics.[i]

4.22 End of life – the period when a patient is living with, and impaired by, a fatal condition, even if the trajectory is ambiguous or unknown. This period may be years in the case of patients with chronic or malignant disease, or very brief in the case of patients who suffer acute and unexpected illnesses or events, such as sepsis, stroke or trauma.[i]

4.23 Evidence-based practice – a process that integrates the best available scientific evidence with professional judgement and patient preferences to make clinical decisions.[xii]

4.24 Guidelines – clinical practice guidelines are systematically developed statements to assist healthcare provider and consumer decisions about appropriate health care for specific circumstances.[i]

4.25 Healthcare provider – an individual who practises a profession relating to the provision of health care. Healthcare providers may be required to maintain profession-specific registration with a national board under the National Registration and Accreditation Scheme or be self-regulated. A healthcare provider may also be referred to as a health practitioner, clinician, or profession-specific description[i]

4.26 Healthcare record – a record of a patient's medical history, treatment notes, observations, correspondence, investigations, test results, prescription records and medication charts for an episode of care.[i]

4.27 Health literacy – Health literacy can be divided into individual health literacy and the health literacy environment. Individual health literacy is the skills, knowledge, motivation and capacity of a person to access, understand, appraise and apply information to make effective decisions about health and health care and take appropriate action. The health literacy environment is the infrastructure, policies, processes, materials, people and relationships that make up the health system and have an impact on the way that people access, understand, appraise and apply health-related information and services.[xiv]

4.28 High-risk medicines – medicines that have an increased risk of causing significant patient harm or death if they are misused or used in error. High-risk medicines may vary between healthcare settings, depending on the types of medicines used and patients treated. Errors with these medicines are not necessarily more common than with other medicines. Because they have a low margin of safety, the consequences of errors with high-risk medicines can be more devastating. At a minimum, the following classes of high-risk medicines should be considered:

- Medicines with a narrow therapeutic index.
- Cytotoxic medicines.
- Medicines that present a high risk when other system errors occur, such as administration via the wrong route.
- Schedule 8 medicines.
- Medicines with a greater potential for addiction or abuse.

4.29 Incident – an event or circumstance that resulted, or could have resulted, in unintended or unnecessary harm to a patient or consumer, or a complaint, loss or damage. An incident may also be a near miss.[i]

4.30 Incident management – the actions taken to address an incident and prevent the incident from recurring. These actions may include conducting a root cause analysis to ascertain the cause of the incident, implementing a corrective action to fix the incident, and taking preventative action to ensure the incident does not occur again.

4.31 Informed consent – a person's voluntary decision about healthcare, made with knowledge and understanding of the benefits and risks involved. [xv]

4.32 Invasive medical devices – devices inserted through skin, mucosal barrier or internal cavity, including central lines, peripheral lines, urinary catheters, chest drains, peripherally inserted central catheters and endotracheal tubes.[i]

4.33 Medical device – therapeutic goods that can be used to diagnose, prevent, treat and monitor medical conditions. They include surgical equipment, syringes, gloves, nebulisers, vaporisers, pacemakers, baby incubators and implants. The Therapeutic Goods Administration (TGA) regulates the supply of medical devices in Australia.[xvi]

4.34 Open disclosure – an open discussion with a patient and carer about an incident that resulted in harm to the patient while receiving health care. The criteria of open disclosure are an expression of regret and a factual explanation of what happened, the potential consequences and the steps taken to manage the event and prevent recurrence.[i]

4.35 Patient – a person who is receiving health care from a healthcare service such as a pharmacy.[i]

4.36 Patient identifiers – items of information for use in identification of a patient, including family and given names, date of birth, gender identity, address, healthcare record number, and individual healthcare identifier.[i]

4.37 Pharmacy – for the purpose of this document, pharmacy refers to a community pharmacy as defined above.

4.38 Pharmacy staff – all people working in a pharmacy including pharmacists, pharmacy technicians, pharmacy assistants and any other people employed or contracted by the pharmacy such as locum pharmacists, third party providers, other healthcare professionals, students or volunteer workers.

4.39 Policy – a course or principle of action adopted or proposed by an organisation or individual.

4.40 Prescribing – an iterative process involving the steps of information gathering, clinical decision-making, communication and evaluation which results in the initiation, continuation or cessation of a medicine.[xvii]

4.41 Procedure – a sequential set of steps which describes a process for completing a task or set of tasks.

4.42 Protocol – an established set of rules used to complete tasks or a set of tasks.[i]

4.43 Quality improvement – the combined efforts of the workforce and others—including people, patients and their families, researchers, planners and educators – to make changes that will lead to better patient outcomes (health), better system performance (care) and better professional development. Quality improvement activities may be undertaken in sequence, intermittently or on a continuous basis.[xii]

4.44 Risk – the chance of something happening that will have a negative impact. Risk is measured by the consequences of an event and its likelihood.[i]

4.45 Risk assessment – assessment, analysis and management of risks. It involves recognising which events may lead to harm in the future and minimising their likelihood and consequences.[i]

4.46 Risk management – the design and implementation of a program to identify and avoid or minimise risks to patients, employees, volunteers, visitors and the organisation.[i]

4.47 Scope of practice (of a pharmacist) – a time-sensitive, dynamic aspect of practice which indicates those professional activities that a pharmacist is educated, competent and authorised to perform and for which they are accountable.[xviii]

4.48 Serious deterioration – physiological, psychological, or cognitive changes that may indicate a worsening of the patient's health status.[i]

4.49 Substitute decision-maker – a person appointed or identified by law to make health, medical, residential and other personal (but not financial or legal) decisions on behalf of a patient whose decision-making capacity is impaired. A substitute decision-maker may be appointed by the patient, appointed for (on behalf of) the person, or identified as the default decision-maker by legislation, which varies by State and Territory.[i]

4.50 System – the resources, policies, processes, and procedures that are organised, integrated, regulated and administered to accomplish a stated goal.[i]

5. Requirements

1. SECTION 1: Governance and Leadership

1.1 Clinical Governance

Pharmacies have governance systems in place that drive continuous improvement of the safety and quality of pharmacy services, ensuring they are person-centred, safe and effective.

Intent: To implement a clinical governance framework that ensures that patients and consumers receive safe and quality health care from pharmacies.

1.1.1 Governance, leadership, and culture

The pharmacy shall set up and use clinical governance systems to improve the safety and quality of health care for patients.

1.1.1.1 The pharmacy shall:

a. Have a culture of safety and quality improvement.
b. Partner with patients, carers, consumers and communities.
c. Establish and maintain systems for integrating care with other service providers involved in a patient's care.
d. Set priorities for safe and quality clinical care and ensure that these are communicated effectively to pharmacy staff.
e. Establish and maintain a clinical governance framework.
f. Clearly define the safety and quality roles, responsibilities and accountabilities of those who have ownership of the pharmacy, management, and pharmacy staff.
g. Monitor and review the safety and quality performance of the pharmacy.
h. Consider the safety and quality of health care for patients in its business decision-making.

1.1.2 Patient safety and quality systems

Safety and quality systems shall be integrated with governance processes to enable the pharmacy to actively manage and improve the safety and quality of health care for patients.

1.1.2.1 To mitigate risk, the pharmacy shall:

a. Establish and maintain policies, procedures and protocols and incorporate these into onboarding of pharmacy staff.
b. Make policies, procedures and protocols easily available to pharmacy staff
c. Monitor and take action to improve adherence to policies, procedures and protocols.
d. Review all policies, procedures and protocols at least annually to ensure they remain fit for purpose.
e. Communicate changes of policies, procedures, and protocols to pharmacy staff.
f. Ensure compliance with relevant safety and quality legislation, regulations and jurisdictional requirements, as well as professional standards, guidelines and codes of practice.

1.1.2.2 The pharmacy shall use a range of information, including from pharmacy staff, patients, consumers and other healthcare providers, to:

a. Identify priorities for safety and quality improvement.
b. Implement and monitor safety and quality improvement activities.
c. Measure changes in safety and quality outcomes.

d. Provide timely information on safety and quality performance to patients, consumers and pharmacy staff.

> NOTE: Safety and quality improvements in pharmacy may encompass clinical and non-clinical activities. Information and its interpretation should be reviewed in collaboration with patients and communities to promote shared decision-making and involvement in care.

1.1.2.3 To manage risks, the pharmacy shall:

a. Support pharmacy staff to identify, mitigate and manage safety and quality risks.
b. Document and routinely monitor safety and quality risks.
c. Plan for, and manage, ongoing service provision during internal and external emergencies and disasters.

1.1.2.4 The pharmacy shall have an incident management system that:

a. Supports pharmacy staff to recognise and report incidents and near misses that occur inside, or outside, the pharmacy.
b. Supports patients, carers, families and other health professionals to communicate concerns or report incidents.
c. Involves pharmacy staff in the review of incidents.
d. Provides timely feedback on the analysis of incidents to pharmacy staff, patients and consumers who have communicated concerns or incidents.
e. Uses the information from the analysis of incidents to improve safety and quality.
f. Incorporates risks identified in the analysis of incidents into the risk management system.
g. Regularly reviews and acts to improve the effectiveness of the incident management and investigation systems.
h. Reports incidents to third parties (for example, indemnity insurers, regulatory bodies, pharmaceutical companies etc.) as appropriate.

> NOTE: The incident management system should focus on improving safety at a system level and should be underpinned by a no-blame approach, allowing pharmacy staff to speak openly about concerns and mistakes.

1.1.2.5 The pharmacy shall use the current Australian Open Disclosure Framework when a patient is harmed through the delivery of health care.

1.1.2.6 The pharmacy shall:

a. Seek feedback from patients and consumers about their experiences and outcomes of health care.
b. Have processes to regularly seek feedback from pharmacy staff on their understanding and use of the safety and quality system.
c. Use feedback to improve safety and quality.

1.1.2.7 The pharmacy shall:

a. Provide opportunities for its patients to report negative feedback.
b. Have processes to address negative feedback in a timely way.
c. Uses information from the analysis of negative feedback to improve safety and quality.

> NOTE: Negative feedback may be received in many ways, including a formal complaint, informal discussion, on social media or in online reviews. The pharmacy should implement a mechanism to flag incidents and feedback as clinical or non-clinical to assist with analysis.

1.1.2.8 The pharmacy shall identify patient populations using its service with distinct needs, priorities and preferences, including:

a. People of Aboriginal and/or Torres Strait Islander origin.
b. People with a disability.
c. People with diverse backgrounds.
d. Other populations and communities identified.

1.1.2.9 The pharmacy shall use information on its patient populations, and collaborate with them, to inform planning and delivery of care that is tailored to their needs and preferences.

Information gathered from patient populations shall be obtained with appropriate consent.

1.1.2.10 The pharmacy shall have a healthcare record system that:

a. Makes patient medication and health records available to pharmacists at the point of dispensing and service delivery.
b. Supports pharmacy staff to maintain accurate and complete records.
c. Complies with privacy and security regulations for the handling, storage and disposal of records.
d. Supports audits of records.
e. Facilitates a patient's access to their healthcare records.

NOTE: Vigilance is required when handling, storing and disposing of all types of records in the pharmacy including, but not limited to, patient healthcare records, employment records and financial records.

1.1.2.11 The pharmacy shall have processes to:

a. Receive and review reports and records on patients (for example, Home Medicines Review reports, hospital discharge summaries, medication charts, pathology reports, real-time prescription monitoring information).
b. Recall patients and communicate about reports, records and health care options.
c. Take action and/or respond to reports and records in a timely manner.
d. Facilitate interprofessional collaboration through sharing reports and records as appropriate.
e. Document reports and records in a patient's healthcare record where possible.

1.1.2.12 Pharmacies using a national electronic health record system (for example, My Health Record) shall have processes to:

a. Use national healthcare identifiers for patients and healthcare providers.
b. Support pharmacists to use the national electronic health record system to optimise the safety and quality of health care for patients.

1.1.2.13 Pharmacies providing clinical information to a national electronic health record system shall have processes to:

a. Comply with legislative requirements, standards, codes and guidelines.
b. Ensure the accuracy and completeness of information uploaded.

1.1.3 Clinical performance and effectiveness

Pharmacy staff shall have the right qualifications, knowledge and skills to provide safe, quality health care to patients.

1.1.3.1 The pharmacy shall:

a. Have processes to support pharmacy staff to understand and fulfil their assigned safety and quality roles and responsibilities.
b. Provide pharmacy staff with orientation and training to their safety and quality roles on commencement with the pharmacy (i.e., onboarding), when safety and quality responsibilities change and when new pharmacy services are introduced.
c. Identify the training needs of pharmacy staff to meet the requirements of this standard.
d. Ensure pharmacy staff complete training to meet their safety and quality training needs.

1.1.3.2 The pharmacy shall support pharmacy staff through training and resources to provide culturally safe services to meet the needs of its Aboriginal and Torres Strait Islander patients and other culturally and linguistically diverse groups.

1.1.3.3 The pharmacy shall have valid and reliable review processes for pharmacy staff that:

a. Are used to regularly review their performance.
b. Identify needs for training and development of safety and quality.

1.1.3.4 The pharmacy shall have processes to ensure that pharmacists have the qualifications, knowledge and skills required to perform their role by:

a. Describing the scope of practice for pharmacists practising in the pharmacy.
b. Monitoring practices to ensure pharmacists are operating within their scope of practice.
c. Reviewing pharmacists' scope of practice when a pharmacy service, procedure or technology is introduced or substantially altered.

1.1.3.5 To support evidence-based care, the pharmacy shall:

a. Provide its pharmacists with ready access to practice guidelines and available evidence, and decision support tools relevant to their clinical practice.
b. Require its pharmacists to use practice guidelines, available evidence and decision support tools relevant to their clinical practice.

1.1.3.6 The pharmacy shall support its pharmacists to:

a. Monitor and review care delivered against relevant evidence.
b. Explore reasons for variation of health care from relevant evidence.
c. Use information on unwarranted variation from evidence-based care to improve health care.

1.1.4 Safe environment for the delivery of care

The environment in which pharmacy services are delivered shall enable safe and quality health care for patients.

1.1.4.1 The pharmacy shall provide safe and appropriate care:

a. Through the design of the environment and management of the location where health care is provided.
b. By appropriately managing, and reviewing, the workload of pharmacy staff.
c. By providing access to an environment, devices and equipment that are fit for purpose and well maintained.
d. By safeguarding patients' privacy when health care is provided.

e. By respecting the cultural needs of patients when health care is provided.

f. By recognising and acknowledging trauma and its prevalence, and being sensitive to its dynamics when health care is provided

g. By taking appropriate action in response to feedback regarding the environment in which health care was provided.

1.1.4.2 The pharmacy shall identify circumstances that have a high risk of potentially causing consumers to behave unpredictably and develop strategies to minimise the risks of harm to patients, consumers and pharmacy staff.

1.1.4.3 The pharmacy shall support all patients to access health care, including patients from diverse backgrounds, patients with a disability and other populations and communities identified.

1.1.4.4 The pharmacy shall provide a culturally safe environment that recognises the importance of the cultural beliefs and practices of Aboriginal and Torres Strait Islander people.

1.1.5 Partnering with patients in their own care

The pharmacy shall develop, implement and maintain systems to partner with patients in their own health care.

1.1.5.1 The pharmacy shall:

a. Use a Charter of Rights consistent with the Australian Charter of Healthcare Rights.

b. Have processes to support pharmacy staff to apply the principles of the Charter of Rights in the planning and delivery of health care.

c. Make the Charter of Rights easily accessible for patients and consumers.

d. Ensure its informed consent processes comply with legislation, Codes of Conduct and the needs of the community.

1.1.5.2 The pharmacy shall have processes to identify:

a. The capacity of a patient to make decisions about their own care.

b. A substitute decision-maker if a patient does not have the capacity to make decisions for themselves.

> **NOTE**: The capacity of a patient to make decisions for themselves can be influenced by their age, language, mental health conditions, disability or other health conditions. Pharmacists should be aware of relevant jurisdictional legislation regarding the age of consent for medical care and should take reasonable steps to determine if a patient's decision-maker has the authority to give consent and make decisions on their behalf. Details of substitute decision-makers should be recorded in the patient's healthcare record.

1.1.5.3 The pharmacy shall have processes for pharmacists to partner with patients and/or their substitute decision-maker to plan, effectively communicate, set and review goals and make decisions about their current and future care.

1.1.5.4 The pharmacy shall support pharmacy staff to form partnerships with patients and consumers so that they can be actively involved in their own health care.

1.1.6 Health literacy

Pharmacy staff shall communicate with consumers in a way that supports effective partnerships.

1.1.6.1 Pharmacy staff shall communicate with patients and consumers about health and health care in a way that:

a. Is tailored to the patient's needs and preferences.
b. Is easily understood.
c. Addresses the need for ongoing health care.

1.1.6.2 The pharmacy shall make information available to consumers on:

a. The services available.
b. Opening hours and how to access health care.
c. Who can access the services.
d. Estimated costs of service delivery.
e. Alternative service when the pharmacy is unavailable.
f. Service location(s) and access details if relevant.
g. Mechanism for providing feedback and contact details for the appropriate healthcare complaints authority.

1.1.7 Partnering with consumers in pharmacy design and governance

Consumers shall be partners in the planning, design, monitoring and evaluation of pharmacy services.

1.1.7.1 The pharmacy shall have a systematic approach to endeavouring to partner with patients, consumers and communities to seek and incorporate their views and experiences into the planning, design, monitoring and evaluation of services.

> NOTE: Co-design and co-development between health professionals, individuals and communities has the potential to build trust, empower individuals and communities and improve health outcomes. Pharmacists should invest in meaningful engagement with patients, carers and communities to design services that have the greatest impact for all.

1.2 Business Governance

Pharmacy owners and managers shall provide a workplace that is safe and secure, complies with legal and professional obligations, provides the infrastructure and training to enable pharmacy staff to deliver the chosen products and services, and has systems in place to ensure viability and sustainability.

Intent: To implement sound business governance practices to ensure professional and statutory obligations are met, workload and risks are managed, and continuity of service is maintained for consumers and the community.

1.2.1 Legal and professional obligations

The pharmacy shall have systems to ensure compliance with all statutory and professional obligations including State and Territory legislation.

1.2.1.1 The pharmacy shall support the provision of the medicines and services it offers by complying with legislative and professional requirements for operating a pharmacy.

1.2.1.2 The pharmacy shall have a systematic approach to ensure each owner and pharmacist employed or contracted by the pharmacy is registered with the Pharmacy Board of Australia.

1.2.1.3 The pharmacy shall ensure all healthcare providers employed or contracted by the pharmacy maintain profession-specific registration as appropriate to their profession.

1.2.1.4 The pharmacy shall maintain appropriate professional indemnity, public liability, and workers' compensation insurance.

1.2.1.5 As an employer, the pharmacy shall comply with its legal responsibilities for employing staff including, but not limited to:

a. Employee payroll tax.
b. Pay rates and conditions.
c. Superannuation payments.
d. Keeping staff records.
e. Workplace health and safety.
f. Leave entitlements.

1.2.1.6 Pharmacy proprietors shall maintain oversight of how their pharmacies are operating.

1.2.2 Business viability

The pharmacy shall have business plans and strategies in place to ensure the pharmacy operates effectively and maintains profitability, viability and sustainability.

1.2.2.1 The pharmacy shall define and document its ownership structure appropriate for the size and type of pharmacy, including:

a. The details of all pharmacy owners, business partnerships, shareholder or unit holders' agreements (as applicable).
b. The details of the pharmacy model and any corporate, banner or not-for-profit entities associated with the pharmacy (as applicable).
c. Reporting requirements to inform all owners of the performance of, and any risks to, the pharmacy business.

1.2.2.2 The pharmacy shall undertake comprehensive business planning, which should be documented and include the following as appropriate for the size and type of pharmacy:

a. A business plan including the dispensary, retail and pharmacy services offered.
b. The resources (for example, funds, finance, staff, stock, technology and equipment) required to execute the business plan.
c. A budget and forecast for the year ahead (at a minimum).

> **NOTE:** Business planning should take into consideration consumer demographics, patient needs, the pharmacy model and structure, and financial and market pressures.

1.2.2.3 At least annually, the following shall be analysed and reported to all pharmacy owners:

a. Total business performance, including any issues identified from its analysis.
b. Financial and business trends.
c. Future financial commitments and budget forecasting.
d. Risks to the viability of the pharmacy business.

> **NOTE**: Other items may also be reported in addition to the above, as determined by the owners and managers of the pharmacy.

1.2.2.4 The pharmacy shall implement strategies to address any issues identified during the analysis of its total business performance to maintain viability.

1.2.2.5 The pharmacy shall consider sustainable business practices that support the business model.

1.2.3 Business continuity plan

The pharmacy shall develop and maintain a business continuity plan for unexpected emergencies such as natural disasters, pandemics, building fires or unplanned absences of key pharmacy staff.

1.2.3.1 The pharmacy shall have a documented business continuity plan which includes strategies and actions to limit the impact to the operations of the pharmacy in an emergency.

1.2.3.2 All pharmacy staff shall be trained in the pharmacy's business continuity plan and understand their role and how to respond in the case of an emergency.

1.2.3.3 The business continuity plan shall be reviewed at least annually, and strategies and actions updated as required.

1.2.4 Pharmacy premises

The pharmacy shall have systems to ensure the pharmacy premises are appropriately maintained.

1.2.4.1 The pharmacy shall:

a. Ensure the pharmacy premises' lease is compliant with the business, and that the business complies with the lease's requirements.
b. If the pharmacy premises are owned, ensure the property is maintained at a standard that is appropriate to allow the safe and professional conduct of the pharmacy business.
c. Not enter, or be bound by, any contracts (including tenancy leases) that restrict the pharmacy's ability to stock products or provide services that meet the therapeutic needs of the community.

1.2.4.2 The pharmacy shall have a Professional Services Area that is distinguishable from the general trading area.

1.2.4.3 The pharmacy shall provide a healthy and safe working environment for pharmacy staff that meets jurisdictional requirements.

> **NOTE:** Pharmacies are required to comply with the mandatory requirements from Safe Work Australia's Compliance Code: Workplace Amenities and Work Environment relevant to their State or Territory.

1.2.4.4 The pharmacy shall maintain a security access register which includes details of who has access to the keys and/or codes to the pharmacy and safes at any given time.

1.2.5 Safety and security

The pharmacy shall have a system to identify and manage potentially dangerous or hazardous situations and emergencies to ensure the safety and security of pharmacy staff, consumers and the business.

1.2.5.1 The pharmacy shall maintain a system to ensure the effective safety of consumers, pharmacy staff and the pharmacy by:

a. Identifying and documenting potential risks and hazards, both physical and psychological.
b. Implementing measures to control, prevent or minimise potential risks and hazards.
c. Following a process to respond to any hazardous situations or emergencies if they occur.
d. Recording and reporting (if appropriate) the details of any hazardous situations or emergencies.
e. Monitoring, reviewing and evaluating preventative measures to ensure they remain effective.

NOTE: Risks and hazards may pose a threat to a person's physical or psychological wellbeing. Employers have a legal responsibility and a duty of care to ensure the health and safety of workers, and to ensure that others are not put at risk from work carried out by a business. While physical injuries are easily recognised, psychological, or psychosocial hazards are often not as obvious. Common psychosocial hazards include role overload, conflict or poor workplace relations, poor support from managers and colleagues, bullying, harassment, racism and exposure to traumatic events.

NOTE: Pharmacies are required to comply with requirements of Safe Work Australia, including Safety Data Sheet requirements for hazardous substances as required.

1.2.6 Information technology (IT)

The pharmacy shall have systems and processes to ensure the appropriate selection, management and accessibility of IT equipment, systems, and data.

1.2.6.1 Owners and/or managers of the pharmacy shall select IT systems that:

a. Are reputable and fit for purpose.
b. Incorporate safeguards against cyber intrusion.
c. Safeguard the integrity and confidentiality of data.
d. Comply with legislative requirements regarding data storage.
e. Protect the availability of the system.

NOTE: IT systems may be located onsite in the pharmacy or off-premises. Regardless of their location, the pharmacy shares the responsibility of ensuring data security with any vendors it engages.

1.2.6.2 Owners and managers of the pharmacy shall maintain relevant IT equipment, systems and communication infrastructure that promote effective and efficient workflows and operations and ensure data security is protected on an ongoing basis.

1.2.6.3 The pharmacy shall proactively prepare a plan to be followed in the event of a data breach or cyber security incident, including a transparent approach to communicating with all stakeholders.

2. SECTION 2: Pharmacy Operations

2.1 Pharmacy Services

Pharmacies shall have systems and processes to ensure patients and consumers receive safe, consistent and quality pharmacy services in compliance with professional standards and guidelines.

Intent: To consistently deliver safe, quality pharmacy services to the community.

2.1.1 Delivery of pharmacy services

The pharmacy shall have systems to ensure programs and services are provided effectively, efficiently and in compliance with professional standards and guidelines, whether supplied within or external to the pharmacy.

2.1.1.1 The pharmacy shall ensure that the staff delivering each program or service are appropriately trained and/or qualified, including that they:

a. Can demonstrate competency, or the clinical knowledge required, to deliver each service.
b. Complete externally provided, Government-endorsed or accredited training where it is available and applicable.
c. Complete training on the pharmacy's procedures for the delivery of individual programs or services.
d. Document completion of the training they have undertaken.

NOTE: Training must be provided to pharmacy staff, not just pharmacists, who are involved in the delivery of pharmacy services. As an example, pharmacy technicians, interns or students may be involved in the delivery of blood pressure monitoring or dose administration aid services and would require appropriate training as outlined above.

2.1.1.2 The pharmacy shall provide access to relevant and current reference material for each program or service it offers.

2.1.1.3 The pharmacy shall ensure it provides a facility or area for the delivery of pharmacy programs or services, and that this facility or area is:

a. Fit for purpose, including the provision of adequate space, light and temperature.
b. Compliant with any legislative or professional practice requirements, including privacy legislation.
c. Cleaned to the level expected of a clinical healthcare destination.
d. Maintained to ensure it remains fit for purpose.

NOTE: Maintenance of a facility or work area may include, but is not limited to, regular servicing of air-conditioning or heating units, inspection and repair of faulty water taps, chairs or privacy screens and replacing light bulbs. Where required, maintenance should be conducted by an appropriately qualified person. Documentation should be kept of maintenance works completed.

2.1.1.4 The pharmacy shall provide any equipment required for a program or service and ensure it is:

a. Fit for purpose.
b. Calibrated, if required.
c. Maintained as specified by the manufacturer.

Records of calibration and maintenance shall be kept by the pharmacy. If external certification of equipment is required, records of this shall also be kept.

2.1.1.5 The pharmacy shall develop and implement a system for the consistent delivery of each program or service that is:

a. Compliant with any legislation, professional practice guidelines, funding or contractual requirements.
b. Provided to all staff members involved in the delivery of each program or service.
c. Reviewed at least annually and updated as required.

2.1.1.6 The pharmacy shall have processes to ensure pharmacists deliver comprehensive care applicable to the services being delivered, by:

a. Conducting a risk screening and assessment.
b. Conducting a clinical assessment and diagnosis as appropriate.
c. Identifying the patient's goals of care.
d. Developing and agreeing a plan for care in partnership with the patient.
e. Delivering comprehensive care in accordance with the agreed plan for health care.
f. Collaborating and communicating with the patient's other healthcare providers as required.
g. Recalling patients for follow-up health care when required.
h. Reviewing and improving the processes of comprehensive care delivery.
i. Receiving a current advance care plan (where one is in place) and incorporating it into a patient's healthcare record.

NOTE: Pharmacists should use their professional judgement to implement the required comprehensive care elements based on the patient's circumstances and the services offered by the pharmacy.

2.1.1.7 The pharmacy shall ensure it has adequate staff available to deliver pharmacy programs and services in addition to other pharmacy operations (such as dispensing prescriptions). The pharmacy shall have a process for prioritising activities if there is a reduction in the number of staff.

2.1.1.8 The pharmacy shall have communication processes to help patients maximise their attendance at planned appointments.

2.1.1.9 The pharmacy shall develop and implement a system to record each episode of care delivered, ensuring that:

a. The records are appropriate for the service that is delivered.
b. The recording occurs in a consistent manner.
c. The records are securely stored and in compliance with legislation.
d. The nature of the records is compliant with relevant professional practice guidelines, funding or contractual requirements.

2.1.1.10 The pharmacy shall develop and implement a process to evaluate the programs and services delivered, including:

a. Monitoring the safety of the programs and services delivered.
b. Assessing the impact of programs or services, where possible.
c. Improving the programs and services based on the outcomes of the evaluation process.

NOTE: The extent of the evaluation process may differ between programs and services.

2.1.2 Prescribing by pharmacists

The pharmacy shall have systems to support the safe and appropriate prescribing of medicines by pharmacists in compliance with professional standards and guidelines.

2.1.2.1 The pharmacy shall maintain and follow a system for prescribing medicines when appropriate and within the scope of practice for a pharmacist.

2.1.2.2 Prescribing services offered at the pharmacy shall comply with relevant professional, legislative and jurisdictional requirements, and support the safe and appropriate use of medicines.

2.1.2.3 The pharmacy shall provide access to, and promote the use of, current, relevant, evidence-based Australian therapeutic guidelines and resources to support prescribing by pharmacists and as required by prescribing protocols or guidelines.

> NOTE: The pharmacy should provide access to additional resources relevant to the therapeutic goods prescribed by pharmacists working in the pharmacy—for example, antimicrobial stewardship standards to support the appropriate use of antimicrobials.

2.1.2.4 The pharmacy shall provide mechanisms for pharmacists to educate consumers about risks and benefits of, and alternatives to, prescribed medicines for their condition.

2.1.2.5 In compliance with relevant Codes of Conduct, the pharmacy shall have a process for declaring conflicts of interests that could affect or could be perceived to affect patient care and shall act in the best interests of patients.

2.1.2.6 The pharmacy shall have a process to evaluate the prescribing services delivered at the pharmacy, including:

a. Monitoring the safety of the prescribing.
b. Compliance with relevant Australian therapeutic guidelines.
c. Assessing the impact of the prescribing, where possible.
d. Improving prescribing services based on the outcomes of the evaluation process where possible.

2.1.3 Advertising of pharmacy services

The pharmacy shall have systems to ensure that advertising of pharmacy services is factual, ethical and complies with relevant codes, guidelines and legislation.

2.1.3.1 The pharmacy shall ensure any clinical or advertising claim is supported by evidence.

2.1.3.2 The pharmacy shall ensure all advertising and promotional material is accurate, balanced, does not mislead and is not capable of leading the consumer to unrealistic expectations.

2.2 Safe and appropriate provision of medicines

The pharmacy shall have systems to ensure therapeutic goods (including compounded medicines) are procured, stored and provided in a manner consistent with legislation, professional standards and guidelines. The systems shall be designed to ensure the most appropriate therapeutic goods are provided and that consumers are given sufficient information to use the medicines safely and effectively.

Intent: To implement processes and procedures to ensure patients and consumers have access to medicines when they need them, and the knowledge to use them safely.

2.2.1 Stock management

The pharmacy shall have systems for ordering, storing, and disposing of therapeutic goods that align with jurisdictional requirements, maintain the integrity of the medicines and support the safe provision of medicines.

2.2.1.1 The pharmacy shall maintain and follow a system for ordering, receiving, unpacking and pricing stock.

2.2.1.2 The pharmacy shall not stock, sell or promote:

a. Tobacco, or tobacco/smoking-related products or implements (this does not refer to therapeutic products used to aid smoking cessation).
b. Alcoholic beverages, home brew or alcohol distilling kits, or products that promote the use of alcohol.
c. Products with no evidence-based therapeutic benefit which do not enhance the practice of pharmacy.

2.2.1.3 The pharmacy shall ensure that no product stocked or supplied by the pharmacy, or used for the delivery of services, is out of date or will become out of date during a normal course of use.

2.2.1.4 The pharmacy shall:

a. Ensure no product supplied, or consumables used for the delivery of services, are subject to a recall.
b. Maintain and follow a system for initiating, managing and reporting products subject to recall.

2.2.1.5 The pharmacy shall comply with professional requirements, manufacturer's instructions, and legislative and jurisdictional requirements for:

a. The identification of high-risk medicines within the pharmacy.
b. Safe and secure storage of medicines, including scheduled medicines, controlled medicines, cytotoxic medicines, medicines identified as high risk and other stock items.
c. The stability of medicines while stored in the pharmacy.
d. Storage of temperature-sensitive medicines and cold chain management.
e. Storage and safe disposal of unused, unwanted or expired medicines including controlled drugs, high-risk medicines, cytotoxic medicines, needles and other sharps.

2.2.1.6 The pharmacy shall ensure it aligns with the National Vaccine Storage Guidelines 'Strive for 5' and other professional guidelines to prevent a cold chain breach of temperature-sensitive medicines.

2.2.2 Safe and appropriate provision of medicines, medical devices and poisons

The pharmacy systems shall be designed to ensure the most appropriate therapeutic goods are recommended and that consumers are provided with sufficient information to use the medicines safely and effectively. Therapeutic goods (including compounded medicines) and poisons shall be provided in accordance with legislative requirements, professional standards and guidelines whether supplied within or external to the pharmacy.

2.2.2.1 The pharmacy shall maintain and follow a system for dispensing prescribed medicines.

2.2.2.2 To support safe and appropriate use, the pharmacy shall comply with professional requirements, manufacturer's instructions and legislative and jurisdictional requirements for the supply of medicines including:

a. Non-prescription medicines.
b. Medicines that require special consideration or are subject to misuse.

c. Medical devices.
d. Poisons and other high-risk products.
e. Medicines packaged in dose administration aids.

2.2.2.3 The pharmacy shall have processes to ensure pharmacists work within their scope of practice to:

a. Take a detailed medication history on presentation or as early as possible in the episode of care.
b. Ensure a patient's medicines-related information is included in a patient's healthcare record.
c. Partner with patients and consumers in the management of their medicines.
d. Support patients and consumers to maintain a current and accurate medicines list.
e. Encourage patients to share their medicines list with other healthcare providers involved in their care and/or does so on a patient's behalf with their consent.
f. Use information on a patient's medication history to minimise risks in the planning and delivery of health care (for example, provision of clinical advice, referral for a medication review or a dose administration aid).

NOTE: Pharmacists may apply a risk-based approach when taking and documenting a medication history as appropriate for each consumer's presentation.

2.2.2.4 The pharmacy shall have processes to ensure pharmacy staff provide information on medicines tailored to the patient's needs and preferences and most appropriate for the medicine being provided.

2.2.2.5 The pharmacy shall maintain and follow a system for simple compounding.

2.2.2.6 The pharmacy shall maintain and follow a system for the management of medication-related problems, which includes:

a. Identifying medication-related problems which may include, but are not limited to, adverse drug reactions, dispensing errors (including near miss events), drug interactions and inappropriate care or advice.
b. Performing an intervention.
c. Documenting the problem and the intervention made.
d. Reviewing medication-related problems and interventions made to identify whether changes in clinical practice or processes are required
e. Taking action to minimise potential medication-related problems.
f. Reporting medication-related problems as required (for example, suspected adverse drug reactions to the Therapeutic Goods Administration, manufacturer, funding provider or other healthcare providers).

2.2.2.7 The pharmacy shall have a system to collaborate with other health care providers to resolve problems relating to medicine use. Details of the collaboration, or attempts to collaborate, shall be documented.

2.2.2.8 The pharmacy shall maintain and follow a system for stock pricing which:

a. Ensures stock prices are consistent with the planned pricing structure and any advertised price.
b. Ensures any displayed or advertised stock price is consistent with the price charged at the point of sale.

2.2.2.9 The pharmacy shall have an effective and efficient process for processing pharmacy sales, including returns and refunds.

2.2.3 Complex compounding (if offered)

The pharmacy shall have systems to ensure that the efficacy, safety and stability of all products prepared in the pharmacy are provided in accordance with legislation, professional standards and guidelines.

2.2.3.1 The pharmacy shall ensure there are appropriately trained and/or qualified staff to prepare compounded medicines, including that they:

a. Can demonstrate competency, or the clinical knowledge required, to prepare compounded medicines.
b. Complete externally provided training where it is available.
c. Complete training on the pharmacy's procedures for the preparation of compounded medicines.
d. Document completion of the training they have undertaken.

NOTE: Training must be provided to all pharmacy staff, not just pharmacists, who are involved in the preparation of compounded medicines.

2.2.3.2 The pharmacy shall ensure they provide access to operating procedures and relevant reference material for all compounds prepared.

2.2.3.3 The pharmacy shall ensure there is a dedicated facility and equipment for complex compounding and a separate dedicated cleanroom for sterile compounding if undertaken.

2.2.3.4 The pharmacy shall maintain and follow a system of risk assessment for workplace health and safety for staff working in the compounding facility.

2.2.3.5 Each time a medicine is compounded, the pharmacy shall follow a system of risk assessment to ensure:

a. Appropriate circumstances for compounding.
b. Patient safety (including taking a medication history).
c. Product formulation.
d. Product quality.

2.2.3.6 The pharmacy shall source high-quality raw materials (pharmaceutical grade where applicable) from reputable suppliers.

2.2.3.7 The pharmacy shall maintain and follow a system to identify, handle, store and dispose of materials safely, both raw and compounded.

2.2.3.8 The pharmacy shall maintain and follow a system to ensure consistency of compounded products, including appropriate instructions for compounding and a system for final product potency testing.

2.2.3.9 The pharmacy shall maintain and follow a system for appropriate packaging, labelling and storage of finished compounded products.

2.2.3.10 The pharmacy shall implement and follow a process for assigning beyond use by dates to ensure efficacy and safety of compounded medicines.

2.2.3.11 The pharmacy shall follow a process for the provision of relevant counselling to patients on the correct use and risks of compounded medicines.

2.2.3.12 The pharmacy shall maintain and follow a system of initiating, managing and reporting a product recall of compounded products.

2.2.4 Indirect provision of medicines

When medicines are provided to consumers and patients without direct interactions with pharmacy staff, the pharmacy shall have systems to ensure that the safety of the patient is maintained and that legal and professional obligations are met.

2.2.4.1 The pharmacy shall maintain and follow a system for the indirect provision of medicines (including compounded medicines), medical devices, and poisons, including:

a. Ordering via internet, post, telehealth and fax.
b. Delivery by a third party on behalf of the pharmacy.
c. Ordering from, and delivery to, other medical facilities such as residential aged care facilities and Aboriginal community health organisations.
d. Product deliveries conducted by pharmacy staff.

A risk assessment shall be conducted for all indirect supply activities and pharmacist involvement (for example, counselling) is still required in line with professional and legislative obligations.

NOTE: When entering contractual arrangements with third party providers, the pharmacy still holds the responsibility for the safe provision of medicines and must not breach legislative or professional requirements. In addition, contractual requirements with third parties (for example Aboriginal Community Controlled Health Organisations) should be reasonable and ethical to best serve patients and communities.

2.2.5 Advertising of therapeutic goods

The pharmacy shall ensure that advertising or promotion of therapeutic goods is factual, ethical and complies with relevant codes, guidelines and legislation.

2.2.5.1 The pharmacy shall ensure any clinical claims:

a. Are consistent with the body of evidence applicable to the products.
b. Are clearly communicated and easily understood by the intended audience.
c. Identify and cite scientific or clinical research appropriately (as applicable).

2.2.5.2 The pharmacy shall ensure that all advertising material includes mandatory statements and is:

a. Accurate.
b. Balanced.
c. Not likely to mislead consumers.
d. Substantiated prior to dissemination.
e. Aligned with information held in the Australian Register of Therapeutic Goods.
f. Not promoting inappropriate or excessive use.
g. Not capable of leading consumers to unrealistic expectations.
h. Not inconsistent with public health campaigns.

2.2.5.3 The pharmacy shall ensure that the dissemination of price information relating to therapeutic goods complies with relevant codes and guidelines.

2.3 Clinical safety

The pharmacy shall have effective systems and processes in place to maximise safe, quality and coordinated care and minimise clinical safety risks such as the spread of infection, serious deterioration in health and self-harm.

Intent: To identify and reduce the likelihood of common clinical safety risks in pharmacy.

2.3.1 Preventing and controlling infections

Evidence-based systems shall be used to prevent and control healthcare-associated infections. Patients presenting with, or with risk factors for, infection or colonisation with an organism of local, national or global significance shall be identified promptly and receive the necessary management and treatment. The pharmacy shall be clean and hygienic.

2.3.1.1 The pharmacy shall have processes to apply standard and transmission-based precautions that are fit for the setting and consistent with current infection control guidelines, jurisdictional requirements and relevant jurisdictional laws and policies, including work health and safety laws.

2.3.1.2 The pharmacy shall have a hand hygiene process that is incorporated in its overarching infection prevention and control program as part of standard precautions and:

a. Is consistent with the National Hand Hygiene Initiative and jurisdictional requirements.
b. Supports pharmacy staff and consumers to practise hand hygiene.

2.3.1.3 The pharmacy shall support staff and consumers to practise respiratory hygiene, cough etiquette and physical distancing.

2.3.1.4 Where aseptic technique is required as part of the provision of health care, the pharmacy shall have processes to:

a. Identify procedures where aseptic technique applies.
b. Monitor healthcare providers' practices to ensure compliance with the pharmacy's policies and procedures on aseptic technique.

> NOTE: Aseptic technique is a set of practices that protect patients and healthcare providers from health care-associated infections and from contact with blood, body fluid and body tissue. Examples of procedures where aseptic technique is required include, but are not limited to, wound dressings, sterile compounding and parenteral medication preparation.

2.3.1.5 Where invasive medical devices are used, the pharmacy shall have processes for the appropriate use and management of invasive medical devices that are consistent with current infection control guidelines.

2.3.1.6 The pharmacy shall have processes to maintain a clean, safe, and hygienic environment—in line with current infection control guidelines, and jurisdictional requirements—to:

a. Respond to environment risks, including novel infections.
b. Require cleaning and disinfection using products listed on the Australian Register of Therapeutic Goods consistent with manufacturers' instructions for use and recommended frequencies.
c. Provide access to training on cleaning processes for routine and outbreak situations and novel infections.

2.3.1.7 The pharmacy shall have processes to evaluate and respond to infection risks for:

a. New and existing equipment, devices and products used in the pharmacy.
b. Clinical and non-clinical areas and workplace amenity areas.
c. Maintaining, repairing and upgrading buildings, equipment, furnishings and fittings (where the pharmacy has authority to do so).
d. Handling, transporting and storage of linen (if applicable).
e. Novel infections and risks identified as part of public health response or pandemic planning.

2.3.1.8 The pharmacy shall have a risked-based pharmacy staff vaccine-preventable diseases screening and immunisation process that:

a. Is consistent with the current edition of the Australian Immunisation Handbook.
b. Is consistent with jurisdictional requirements for vaccine-preventable diseases.
c. Identifies and addresses specific risks to pharmacy staff, consumers and patients.

2.3.1.9 The pharmacy shall have risk-based processes for preventing and managing infections among pharmacy staff that:

a. Are consistent with the relevant State or Territory work health safety regulation and current infection control guidelines.
b. Align with State and Territory public health requirements for pharmacy staff screening and exclusion periods.
c. Manage risks to pharmacy staff, patients and visitors, including for novel infections.
d. Promote non-attendance or remote attendance at work where possible when infection is present or suspected. Where non-attendance is unavoidable, appropriate personal protective equipment (PPE) should be used.
e. Plan for, and manage, ongoing service provision during outbreaks and pandemics or events where there is increased risk of transmission of infection.

2.3.1.10 Where reusable equipment, instruments and devices are used, the pharmacy shall have:

a. Processes for reprocessing that are consistent with relevant national or international standards, in conjunction with manufacturers' guidelines.
b. A process for critical equipment, instruments and devices that can identify:
    i. The patient.
    ii. The procedure.
    iii. The reusable equipment, instruments and devices that were used for the procedure.
c. Processes to plan and manage reprocessing requirements and additional controls for novel and emerging infections.

2.3.2 Comprehensive care

The pharmacy shall deliver coordinated care that is clinically appropriate and takes into consideration the patient's preferences. This care shall be planned, delivered in collaboration with the patient, and consider the effect of the patient's health issues on their life and wellbeing. Comprehensive care may be a discrete episode of care or part of an ongoing comprehensive care plan and be within the context of clinical services provided by the pharmacy.

2.3.2.1 The pharmacy shall have a system to:

a. Collaborate with other healthcare providers involved in a patient's care.
b. Support collaboration with other care providers to develop a coordinated approach to the planning and delivery of health care.
c. Facilitate reporting to a patient's other relevant care providers.
d. Record collaboration, or attempts to collaborate, with other healthcare providers.

2.3.2.2 The pharmacy shall have processes to support health education and promotion, illness prevention and early intervention for patients, considering its patient population.

2.3.2.3 The pharmacy shall have processes to:

a. Check if a patient is of Aboriginal and/or Torres Strait Islander origin for services or settings where this is relevant or appropriate (e.g., immunisations).
b. Record this information in a relevant part of the patient's healthcare record (e.g., Australian Immunisation Register).
c. Use this information to optimise the planning and delivery of health care.

2.3.2.4 The pharmacy shall support its staff to meet the individual needs of its patients, including those:

a. With a disability.
b. From diverse populations.
c. Who are receiving palliative or end of life care.

2.3.3 Communicating for safety

The pharmacy shall adopt timely, purpose-driven and effective communication strategies that support continuous, coordinated and safe care for patients.

2.3.3.1 The pharmacy shall have processes that use at least three patient identifiers to ensure patients are correctly identified.

NOTE: Correctly identifying patients is particularly important when handing out prescription medications or dose administration aids.

2.3.3.2 The pharmacy shall have processes to:

a. Correctly match patients to their care
b. Ensure essential information is documented in a patient's healthcare record.

2.3.3.3 The pharmacy shall support its pharmacists to refer patients to other services and collaborate with other care providers by:

a. Using structured communication processes.
b. Considering the patient's risks, goals and preferences for health care.
c. Communicating information that is current, comprehensive and accurate.

2.3.3.4 The pharmacy shall use its communication processes to effectively communicate critical information, alerts and risks in a timely way when they emerge or change to:

a. Relevant healthcare providers involved in the patient's care.
b. Patients, carers and families, in accordance with the patient's preferences.

2.3.3.5 The pharmacy shall have communication processes for patients and consumers to directly communicate critical information and risks about care to their healthcare providers.

2.3.4 Recognising and responding to serious deterioration and minimising harm

The pharmacy shall have systems in place to recognise and respond to acute deterioration in patients and escalate care appropriately.

2.3.4.1 Pharmacists shall use the pharmacy's processes to:

a. Recognise deterioration in a patient's physical, mental or cognitive health.
b. Respond to a patient within their scope of practice and call for emergency assistance.
c. Notify a patient's other relevant healthcare providers, carers or family when their health care is escalated.

2.3.4.2 The pharmacy shall:

a. Have processes to respond to patients who are distressed, have expressed thoughts of self-harm or suicide, or have self-harmed.
b. Have processes to respond to patients who present a risk of harm to others.
c. Provide information on accessing other services to patients with healthcare needs beyond the scope of the pharmacy.
d. Have a process that supports crisis intervention that is aligned to legislation

2.4 Business Operations

The pharmacy shall have systems and processes to ensure the effective operation of the pharmacy including the recruitment, oversight and training of pharmacy staff, and the identification and prevention of risks to the pharmacy business.

Intent: To implement processes and procedures to support the daily operation of the pharmacy.

2.4.1 Human resources

The pharmacy shall have processes to employ, manage, train and develop pharmacy staff, as well as to manage voluntary and involuntary departures of members of the pharmacy staff.

2.4.1.1 The pharmacy shall have a process for the recruitment of members of the pharmacy staff that includes:

a. A recruitment procedure.
b. Developing position descriptions for all positions and using these as the basis for any selection process.
c. Maintaining records of interviews conducted with short-listed applicants.
d. Providing successful applicants with a written contract of employment defining the terms and conditions of engagement, prior to commencing employment.
e. Collecting and storing a signed copy of the contract of employment for all successful applicants.
f. Advising unsuccessful applicants that they have not been selected for employment.

2.4.1.2 The pharmacy shall ensure there is a formal agreement in place to support pharmacy students undertaking work integrated placements in the pharmacy.

> NOTE: Universities and Vocational Education and Training (VET) providers should provide a formal agreement template during the process of establishing a student's placement. Pharmacies should also confirm pharmacy-appropriate professional indemnity arrangements are in place for students undertaking placements in the pharmacy.

2.4.1.3 The pharmacy shall have processes for the oversight, supervision and management of members of the pharmacy staff which include:

a. An induction process that is completed within three months of a new member of the pharmacy staff commencing employment.
b. Assessing that new members of the pharmacy staff can apply all policies and procedures applicable to their role as outlined in the induction process.
c. Supervising and managing the performance of new members of the pharmacy staff during their qualifying/probationary period.
d. A performance review process to monitor, inform and manage pharmacy staff performance.

e. Maintaining a pharmacy staff roster that provides sufficient capability to support the safe and appropriate provision of medicines and pharmacy services and is compliant with relevant industrial instruments (for example the Pharmacy Industry Award).
f. Seeking feedback from pharmacy staff on issues affecting them and/or their work environment.
g. A formal workplace grievance process that is utilised when required.
h. Developing and maintaining a personnel file for each member of the pharmacy staff, ensuring confidentiality and privacy of all personnel information.

2.4.1.4 The pharmacy shall implement a process to oversee providers contracted by a third party to deliver a health service on behalf of the pharmacy.

2.4.1.5 The pharmacy shall systematically plan and deliver ongoing training for pharmacy staff by:
a. Maintaining a training and development plan for each member of the pharmacy staff which outlines their training needs relevant to the positions they hold.
b. Providing resources and opportunities for members of the pharmacy staff to meet their identified training needs.
c. Providing members of the pharmacy staff with continuous access to training on products, services and procedures relevant to the pharmacy's business model and the positions they hold.
d. Maintaining a training record for each member of the pharmacy staff that details all training offered and undertaken.

2.4.1.6 The pharmacy shall have a process to manage dismissals and resignations of members of the pharmacy staff that includes:
a. Recording the reasons for dismissals or resignations.
b. Offering all departing members of the pharmacy staff an exit interview, recording that the offer was made and keeping records of the interview in their personnel file.
c. Offering departing members of the pharmacy staff appropriate final documents including a statement of service and a copy of their training and development record.
d. Retaining records of dismissals and resignations for the required period in accordance with legislation.

2.4.1.7 Where possible, the pharmacy should utilise feedback provided by departing members of the pharmacy staff to make improvements to the operation of the pharmacy.

2.4.2 Safety and security

The pharmacy shall have processes to educate pharmacy staff of potentially dangerous situations and emergencies, facilitate reporting of hazardous situations and ensure pharmacy staff have appropriate qualifications to assist in the event of a medical emergency.

2.4.2.1 All members of the pharmacy staff shall be educated on:
a. Potential risks and hazards in the pharmacy and their role in controlling, preventing and minimising them.
b. How to report risks, dangerous and hazardous situations that may arise.

2.4.2.2 The pharmacy shall ensure there are members of the pharmacy staff with appropriate and current qualifications in First Aid and CPR as required for the services the pharmacy provides.

The pharmacy should have access to a defibrillator and ensure pharmacy staff are able to use the defibrillator in the case of an emergency.

2.4.3 Information technology (IT)

The pharmacy shall have systems and processes to ensure the appropriate access to IT equipment, systems and data and to prevent breaches in data privacy and loss of system access.

2.4.3.1 The pharmacy shall promote a culture of cyber resilience by:

a. Developing a cyber strategy, proactively overseen by the pharmacy owners and managers.
b. Setting clear roles and responsibilities in relation to cyber resilience.
c. Training pharmacy staff to ensure they have the knowledge and skills to utilise the IT systems in place and that they understand the importance of data privacy and cybersecurity.
d. Promoting strong cyber security practices.

2.4.3.2 The pharmacy shall implement and maintain processes to ensure:

a. Access to IT systems and data is restricted to authorised personnel.
b. There is no unauthorised or unsupervised access to confidential data held or accessed by the pharmacy.

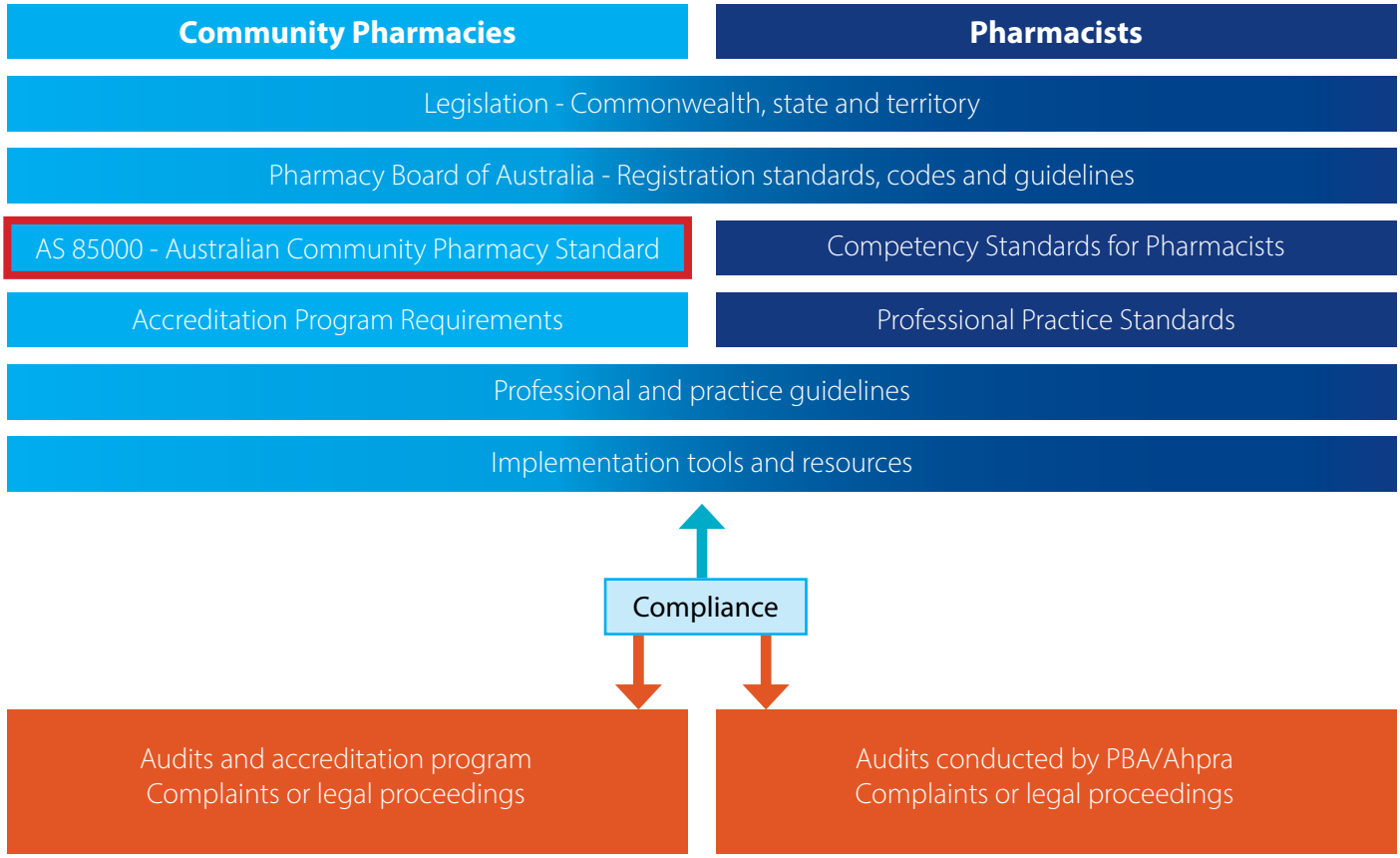
2.4.3.3 The pharmacy shall implement, maintain and follow a process to recover and restore IT systems and data in the event of infrastructure loss, or loss of system access.

6. Appendices

Relationship between key pharmacy publications

Community pharmacy practice, and the practice of pharmacists, are underpinned by many layers of legislation, standards, codes and professional documents. While some guidance documents contain information specific to pharmacists and their roles as healthcare providers, others cover requirements for community pharmacies as healthcare destinations. There are also documents which support both.

The diagram below shows key pharmacy publications that guide pharmacists in their individual practice, those which outline the requirements for community pharmacies, and those which contain parameters for both. Methods of assessing how pharmacies and pharmacists conform with acceptable standards of practice is also shown.



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